TABLE 3. Recommendations for regimens to treat latent tuberculosis infection

Priority rank*	Regimen	Recommendation (strong or conditional)	Evidence (high, moderate, low, or very low)
Preferred	3 mos isoniazid plus rifapentine given once weekly	Strong	Moderate
Preferred	4 mos rifampin given daily	Strong	Moderate (HIV negative)†
Preferred	3 mos isoniazid plus rifampin given daily	Conditional	Very low (HIV negative)
		Conditional	Low (HIV positive)
Alternative	6 mos isoniazid given daily	Strong§	Moderate (HIV negative)
		Conditional	Moderate (HIV positive)
Alternative	9 mos isoniazid given daily	Conditional	Moderate

Abbreviation: HIV = human immunodeficiency virus.
* *Preferred:* excellent tolerability and efficacy, shorter treatment duration, higher completion rates than longer regimens and therefore higher effectiveness; *alternative:* excellent efficacy but concerns regarding longer treatment duration, lower completion rates, and therefore lower effectiveness.
† No evidence reported in HIV-positive persons.
§ Strong recommendation for those persons unable to take a preferred regimen (e.g., due to drug intolerability or drug-drug interactions).

TABLE 4. Dosages for recommended latent tuberculosis infection treatment regimens

Drug	Duration	Dose and age group	Frequency	Total doses
Isoniazid* and rifapentine†	3 mos	**Adults and children aged ≥12 yrs** Isoniazid: 15 mg/kg rounded up to the nearest 50 or 100 mg; 900 mg maximum Rifapentine: 10–14.0 kg, 300 mg 14.1–25.0 kg, 450 mg 25.1–32.0 kg, 600 mg 32.1–49.9 kg, 750 mg ≥50.0 kg, 900 mg maximum **Children aged 2–11 yrs** Isoniazid*: 25 mg/kg; 900 mg maximum Rifapentine†: see above	Once weekly	12
Rifampin¶	4 mos	**Adults:** 10 mg/kg **Children:** 15–20 mg/kg** **Maximum dose:** 600 mg	Daily	120
Isoniazid* and rifampin¶	3 mos	**Adults** Isoniazid*: 5 mg/kg; 300 mg maximum Rifampin¶: 10 mg/kg; 600 mg maximum **Children** Isoniazid*: 10–20 mg/kg††; 300 mg maximum Rifampin¶: 15–20 mg/kg; 600 mg maximum	Daily	90
Isoniazid*	6 mos	**Adults:** 5 mg/kg **Children:** 10–20 mg/kg†† **Maximum dose:** 300 mg	Daily	180
		Adults:15 mg/kg **Children:** 20–40 mg/kg†† **Maximum dose:** 900 mg	Twice weekly§	52
	9 mos	**Adults:** 5 mg/kg **Children:** 10–20 mg/kg†† **Maximum dose:** 300 mg	Daily	270
		Adults: 15 mg/kg **Children:** 20–40 mg/kg†† **Maximum dose:** 900 mg	Twice weekly§	76

* Isoniazid is formulated as 100-mg and 300-mg tablets.
† Rifapentine is formulated as 150-mg tablets in blister packs that should be kept sealed until use.
§ Intermittent regimens must be provided via directly observed therapy (i.e., a health care worker observes the ingestion of medication).
¶ Rifampin (rifampicin) is formulated as 150-mg and 300-mg capsules.
** The American Academy of Pediatrics acknowledges that some experts use rifampin at 20–30 mg/kg for the daily regimen when prescribing for infants and toddlers (**Source:** American Academy of Pediatrics. Tuberculosis. In: Kimberlin DW, Brady MT, Jackson MA, Long SS, eds. Red Book: 2018 Report of the Committee on Infectious Diseases. 31st ed. Itasca, IL: American Academy of Pediatrics; 2018:829–53).
†† The American Academy of Pediatrics recommends an isoniazid dosage of 10–15 mg/kg for the daily regimen and 20–30 mg/kg for the twice-weekly regimen.

for asymptomatic or subclinical TB disease increases, possibly facilitating rifampin resistance if TB disease is inadvertently treated with rifampin monotherapy (*89*).

Three Months of Daily Isoniazid Plus Rifampin

A regimen of 3 months of daily isoniazid plus rifampin is a preferred treatment that is conditionally recommended for adults and children of all ages and for HIV-positive persons as